

DEPARTMENT OF HEALTH AND HUMAN SERVICES — FOOD AND DRUG ADMINISTRATION
MEDWATCH: FORM FDA 3500A (10/15) — MANDATORY ADVERSE EVENT REPORTING
FOLLOW-UP REPORT #1 (CLINICAL RESOLUTION & DECHALLENGE CONFIRMATION)
Original Case Reference: CR-2025-US-00744 | OMB Control No. 0910-0291

SECTION A: PATIENT IDENTIFIER

1. PATIENT IDENTIFIER: D.M. (David Miller)	2. AGE: 52 YRS	2a. DOB: 11-FEB-1973	3. SEX: MALE	4. WEIGHT: 79 kg
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SECTION B: FOLLOW-UP EVENT DESCRIPTION & CLINICAL RESOLUTION

FOLLOW-UP CLINICAL COURSE (14-DAY POST-DISCONTINUATION REVIEW):

Initial report dated 02-NOV-2025 documented a witnessed generalized tonic-clonic seizure occurring 48 hours following rapid titration of Neuroval (neuroval hydrochloride) from 200 mg to 400 mg daily for neuropathic pain. Patient was admitted emergently to Columbia University Neurological ICU on 02-NOV-2025.

DECHALLENGE CONFIRMATION: Neuroval was permanently withdrawn on 02-NOV-2025. Follow-up evaluation on 16-NOV-2025 confirms the patient has remained entirely seizure-free for 14 consecutive days without requiring ongoing anticonvulsant therapy. Repeat 24-hour ambulatory video EEG completed on 14-NOV-2025 demonstrated normalization of cerebral background activity with zero epileptiform spike-wave discharges. Brain MRI with contrast showed no structural epileptogenic focus. **Final Case Outcome: FULLY RECOVERED / RESOLVED. Causality: Probable related to drug exposure.**

SECTION C: SUSPECT PRODUCT DETAILS

PRODUCT: Neuroval (neuroval HCl) 200mg/400mg	DOSE: Titrated to 400 mg PO QD	LOT #: Lot #NV-2025-110 (Exp 09/27)
THERAPY DATES: 15-OCT-2025 to 02-NOV-2025	DECHALLENGE: [X] YES — Event abated after withdrawal	RECHALLENGE: [X] NOT DONE

SECTION E: INITIAL / FOLLOW-UP REPORTER

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